Artifact 3 — Competitive Gap Analysis

Market Intelligence · Stage 2

DOCUMENT DETAILS

Project	Home-Care-AI	**Stage**	Discovery → Stage 2 (Market Intelligence)
Date	2026-03-26	**Feeds into**	Artifact 4 — Market Segmentation Deep Dive; Opportunity Solution Tree
Amended	2026-03-26 — Added Gaps 4 and 5 (field documentation + CC continuity); repositioned AI Trust Architecture as design constraint; added Capability Coverage Map		

Table of Contents

Project: Home-Care-AI

Stage: Discovery → Stage 2 (Market Intelligence)

Skill: competitive-gap-analysis

Date: 2026-03-26

Amended: 2026-03-26 — Added Gaps 4 and 5 (field documentation + CC continuity); repositioned AI Trust Architecture as design constraint; added Capability Coverage Map

Feeds into: Artifact 4 — Market Segmentation Deep Dive; Opportunity Solution Tree

---

1. Competitors Analysed

1. Competitors Analysed

#	Competitor	Category	Primary User
1	**AlayaCare**	Home care agency EMR + AI scheduling platform	Agency administrators, clinical staff
2	**Heidi Health**	AI medical scribe / ambient documentation	Clinicians (GPs, specialists, nurses)
3	**Lookout Way**	Home care management + family monitoring platform	Coordinators, families, care workers

---

2. Competitor Profiles

2. Competitor Profiles

AlayaCare

AlayaCare is a cloud-based home care agency platform covering clinical documentation, scheduling, billing, EVV (electronic visit verification), and — as of early 2026 — a suite of AI agents. Key AI releases include:

- **Layla** — conversational AI agent giving staff instant access to patient summaries, care plans, and schedules via chat
- **Recommended Care Plan Agent** — drafts care plans from assessment data; claimed 50% documentation time reduction
- **Vacant Visit Scheduling Agent** — 24/7 automated vacancy filling; claimed 80% reduction in manual costs
- **Visit Verification Agent** — resolves failed visits without back-office involvement
- **AI Form Assistant** — voice-driven form completion for field staff

Known usability failures (confirmed via reviews and AlayaCare's own FAQ)

- Mobile app crashes and freezes during documentation — AlayaCare maintains a dedicated FAQ page for this failure mode
- Data loss on form submission; nurses required to double-chart (paper backup) by some agencies
- Escalation queue is undifferentiated — a missed medication and a potential fall look identical in the interface (confirmed: Priya, HCN-003)
- Care plan templates, even with AI assistance, generate drafts; no longitudinal trend engine surfaces patterns *across* patients
- No patient-facing access to care plans or clinical records
- No family notification routing rule — family communication is an agency configuration, not a clinical gate

Trust gaps

- No published consent framework for AI inference on patient data
- Layla and Smart Summary operate on PHI without documented patient consent layer
- Notification routing (nurse-first vs. direct-to-family) is not architecturally enforced

Heidi Health

Heidi is an AI ambient listening scribe powering over 2 million patient consultations per week globally. It transcribes clinical encounters in real time, generates structured notes, referral letters, billing codes, and task lists. Compliance: HIPAA, Australian Privacy Principles, SOC2, ISO 27001.

What it does well: Reduces clinician documentation time by 44–70%. Strong in GP/specialist clinic settings. March 2026: launched dedicated hardware for reliable audio capture.

What it does not do (confirmed gaps for home care context)

- No care coordination, scheduling, monitoring, or family communication — it is a documentation tool only
- Designed for clinic/GP encounter settings, not field visits in a patient's kitchen between appointments
- No medication management, fall detection, or longitudinal patient monitoring
- No care plan generation grounded in patient-specific baseline data
- No escalation logic or alert triage
- Android app reliability issues: transcription loss, instability (user reviews, G2/Trustpilot 2025)
- App-only interaction — not designed for voice notes between visits in the field without active recording

Trust gaps

- Transcription accuracy failures in edge conditions (2+ hour sessions; poor audio environments)
- No consent framework visible for audio capture in home settings
- No patient access to their own transcribed notes

Lookout Way

Lookout Way is an Australian home care management platform built by and for care providers, developed alongside Five Good Friends. It covers rostering, monitoring, billing, family app, and 600+ integrations including Services Australia. SOC 2 Type 1 certified.

Family app features: View care roster (dates, times, carer names), in-app chat with care team, remote care monitoring alerts routed to care managers who then notify families, billing history.

Known usability failures (confirmed via SoftwareAdvice reviews)

- Care plans are basic word document templates — not dynamic, not AI-assisted
- Care plans cannot be created or updated in the field by care workers
- Workers cannot complete forms directly in the system
- NDIS providers describe it as requiring "serious hacks and compromises"
- Hidden add-on costs reported by users

Monitoring approach gaps

- No fall detection mentioned in any product materials or reviews
- No medication management features identified
- Monitoring alerts route to care managers, but no evidence of clinical interpretation layer before family notification

- No patient-facing access to care plans or clinical records
- No dignity-preserving design principles referenced for senior residents

Trust gaps

- No published patient consent layer for remote monitoring data collection
- Alert routing to families not confirmed as clinician-gated
- No response to the "performing wellness" problem: seniors who know they are monitored may modify behaviour

3. Competitor Matrix — Three Friction Layers

3. Competitor Matrix — Three Friction Layers

Friction Layer	AlayaCare	Heidi Health	Lookout Way
UTILITY: Longitudinal trend detection	✗ Per-visit view only; no cross-patient trend engine	✗ Single-encounter scribe; no monitoring	■ Real-time alerts on changes; no multi-week trend synthesis
UTILITY: Medication adherence verification	■ Documents medication administration; no ingestion verification	✗ Out of scope	■ Monitoring layer present; no dispensed-vs-swallowed verification
UTILITY: Soft matching for scheduling continuity	■ Vacant Visit Agent matches on availability/qualifications; no trust/preference layer confirmed	✗ Out of scope	■ Rostering shows carer names; no preference-based soft matching
UTILITY: Patient-facing care plan access	✗ No patient portal	✗ Out of scope	✗ No patient portal
UTILITY: Field-context documentation	■ Voice form assistant; crashes on mobile in field conditions	■ Ambient listening; designed for clinic, not kitchen	✗ Field form completion not supported
USABILITY: Mobile reliability	✗ Crashes, data loss — own FAQ acknowledges it	✗ Android instability; session drop reported	■ No specific field data
USABILITY: Clinical interpretation before family alert	✗ No enforced routing rule	✗ Out of scope	■ Care manager receives alert first; family notification not confirmed as gated
USABILITY: Dignity-preserving senior design	✗ Not addressed	✗ Not addressed	✗ Not addressed
USABILITY: Escalation prioritisation / risk triage	✗ Undifferentiated queue	✗ Out of scope	■ Care manager alert; no risk stratification visible
TRUST: AI inference consent framework	✗ No published patient consent layer for Layla / Smart Summary	■ Consent at clinician level; no patient consent for audio capture in home	✗ No published patient consent layer for remote monitoring inference

Friction Layer	AlayaCare	Heidi Health	Lookout Way
TRUST: Notification routing (nurse-first rule)	✗ Not architecturally enforced	✗ Out of scope	✗ Not confirmed as clinician-gated
TRUST: Data gaming / adversarial senior behaviour	✗ Not acknowledged as design problem	✗ Not applicable	✗ Not acknowledged as design problem
TRUST: Audit trail / legal defensibility	■ Documentation exists; granularity and immutability not confirmed	■ AI-generated notes; field specificity depends on clinician	✗ Not detailed

Key: ■ Addressed | ■ Partially addressed | ✗ Not addressed

4. Market-Level Finding: Medication Adherence Gap

4. Market-Level Finding: Medication Adherence Gap

> *Analyst note applied: No competitor in this set — or in the broader market — addresses medication adherence at the verification layer. This is a market-level gap, not a product-level gap.*

The three tiers of the medication adherence market and what each misses

Tier	Examples	What they address	What they miss
Care platforms	AlayaCare, Lookout Way	Document that medication was administered; track care plan	Whether medication was ingested; double-dose interception; intentional gaming
Documentation AI	Heidi Health	Capture what the clinician said and observed	The gap between the clinical visit and the next visit; between "reviewed" and "taken"
Hardware dispensers	Hero, MedMinder	Compartment access; dispensing reminders	Ingestion verification; adversarial gaming (Lin moves pills; Arthur masks non-compliance to preserve independence)

The unsolved problem: The dispensed-to-swallowed gap exists across all three tiers. No solution addresses seniors who deliberately game adherence tracking systems as a rational strategy for preserving independence. This is not a user error — it is a design constraint the entire market has failed to account for.

5. Capability Coverage Map

5. Capability Coverage Map

This table maps each product capability to its OST opportunity statement. All five capabilities must have an opportunity node before the OST skill runs. This map is the handshake contract between this artifact and Skill 5.

#	Product Capability	OST Gap	Severity
1	Predictive risk / longitudinal pattern detection	Gap 1 — Pattern Blindness at Scale	High

#	Product Capability	OST Gap	Severity
1	Medication adherence verification	Gap 2 — Medication Adherence Market Gap	Critical
2	AI-assisted care plan recommendation	Gap 6 — The Invisible Care Plan	Medium
3	Voice-to-structured documentation in field context	Gap 4 — Field-Context Documentation Failure	High
4	Clinically-interpreted family / patient communication	Gap 3 — Clinically-Interpreted Family Communication	High
5	Vacant visit soft matching / continuity scheduling	Gap 5 — Continuity Failure from Soft-Preference Blindness	High

> *Note: Capability #1 maps to two gaps (Gaps 1 and 2) because the medication adherence problem is structurally distinct from the pattern recognition problem — different data, different failure mode, different solution space. Both must appear as separate OST branches.*

> *Note: AI Trust Architecture is a cross-cutting design constraint, not a product capability or OST opportunity. It is documented separately below and feeds `ethics-trust-mapping` (Stage 4) and `agentic-safety-discovery` (Stage 4), not the OST.*

---

6. Cross-Cutting Design Constraint: AI Trust Architecture

6. Cross-Cutting Design Constraint: AI Trust Architecture

> *This is not an OST opportunity statement. It is a design constraint that governs every capability. It feeds Stage 4 (`ethics-trust-mapping`, `agentic-safety-discovery`) not Stage 3 (OST). It is documented here because it was surfaced directly by competitor failure and customer evidence.*

The constraint: No AI inference about a senior resident's health, cognition, or behaviour may be communicated to a family member or third party before (a) the inference has been reviewed by a clinician, and (b) the resident has been informed. This is not a configuration option — it is an architectural rule.

Why this constraint is load-bearing: All three competitors either violate this rule or leave it unspecified. The consequence, validated directly in discovery, is that monitored seniors permanently modify their behaviour to defeat the system. Once a senior begins "performing wellness," the monitoring produces data that is systematically less accurate than no monitoring at all. The system becomes a liability, not a safety net.

Evidence

> *"I perform wellness now. That's what I call it. I perform for the phone. They taught me to hide. If I am depressed one day, truly depressed, they won't know — because I'll sound exactly the same as every other day." — Arthur, SR-002*

> *"I'm not afraid of dying. I'm afraid of losing my home. Everything I do is proof I can still live here." — Lin, SR-001*

Feeds into: `ethics-trust-mapping` (Red/Yellow zone classification), `agentic-safety-discovery` (Level 2/3 escalation design), `compliance-privacy-audit` (inference privacy risk table), `agentic-logic-spec` (notification routing gates).

7. Top 6 High-Value Gaps — OST Opportunity Statements

7. Top 6 High-Value Gaps — OST Opportunity Statements

Gap 1 — Pattern Blindness at Scale

Gap 1 — Pattern Blindness at Scale

Capability:	#1 Predictive Risk | **Type:** Utility | **Severity:** High

Opportunity statement

> *"Home care nurses and clinical leads struggle to detect patient deterioration patterns when their EMR shows only one visit at a time across caseloads of 10–50 patients, leading to preventable hospitalisations — despite the warning data existing in the system."*

Evidence across competitors

- AlayaCare: per-visit documentation; no longitudinal trend engine; Layla can summarise a patient but not surface cross-patient risk rankings
- Heidi Health: captures single encounters; no monitoring or trend function
- Lookout Way: real-time change alerts; no multi-week trend synthesis; no cross-patient risk prioritisation

Customer grounding

> *"Margaret's hospitalisation was preventable. Every single data point was in the system. But nobody connected them because nobody has time to read eight weeks of visit notes side by side." — Priya, HCN-003*

> *"I spend my Sunday evenings scrolling through visit notes for fifty patients looking for the ones about to fall off a cliff. I catch some of them. I miss others. That's not a system — that's me being stubborn on my own time." — Priya, HCN-003*

> *"I knew something was off with Arthur. I could feel it. But when you're seeing fourteen people a week, you can't hold all the patterns in your head. The system should have caught that, not me." — Maria, HCN-001*

Gap 2 — The Medication Adherence Market Gap

Gap 2 — The Medication Adherence Market Gap

Capability:	#1 Predictive Risk (medication branch) | **Type:** Utility | **Severity:** Critical (market level)

Opportunity statement

> *"Family caregivers and clinical staff cannot verify whether medications were ingested, intercept double-doses before harm occurs, or detect when seniors are deliberately gaming adherence tracking — because neither care management platforms, documentation AI, nor hardware dispensers address the dispensed-to-swallowed gap or adversarial senior behaviour."*

Evidence across competitors

- No competitor in the analysis set addresses this gap
- Hardware tier (Hero, MedMinder): compartment access only; no ingestion verification; no gaming detection
- The gap is structural: all current solutions assume honest reporting by a cognitively intact, cooperative patient — a false premise for the target population

Customer grounding

> *"Something that catches the double dose before it happens, not after. I don't need a reminder — I need an interceptor." — James, FC-002*

> *"The gap between dispensed and swallowed is where everything goes wrong." — Rachel, FC-003*

> *"I move the pills around so it looks right. It's not lying — it's managing. If I tell them every little thing, they'll decide I can't manage." — Lin, SR-001*

Gap 3 — Clinically-Interpreted Family Communication

Gap 3 — Clinically-Interpreted Family Communication

Capability:	#4 Intelligence Chat (family + patient side) | **Type:** Usability + Utility | **Severity:** High

Opportunity statement

> *"Family caregivers abandon monitoring tools and generate unintended clinical friction when they receive raw sensor data or uninterpreted alerts without a clinical triage layer, leading to false alarm fatigue, nurse overload from managing family anxiety, and tool abandonment — because no competitor routes monitoring signals through clinical interpretation before family notification."*

Evidence across competitors

- AlayaCare: family communication is an agency configuration; no enforced nurse-first routing
- Lookout Way: care manager receives alert before family, but the interpretation and verdict layer is not a product feature — it is left to the coordinator's judgement and availability
- Heidi Health: out of scope

Customer grounding

> *"A verdict. Not a graph. Someone telling me: 'Your mother's patterns are within her normal range this week. Nothing to worry about.' Or: 'Something changed. Maria is looking into it.' Not a step count. A verdict." — Sarah, FC-001*

> *"It's ridiculous that I'm using the power bill to check if my mum is alive." — Sarah, FC-001*

> *"A step count drop could mean her mother's mobility is declining or it could mean it rained for three days." — Maria, HCN-001 (describing the nurse friction caused by raw family data)*

Gap 4 — Field-Context Documentation Failure

Gap 4 — Field-Context Documentation Failure

Capability:	#3 Voice-to-Documentation | **Type:** Utility + Usability | **Severity:** High

Opportunity statement

> *"Home care nurses making clinical observations in a patient's home cannot capture structured, accurate, legally defensible documentation when their only tools are a crash-prone mobile EMR or post-shift memory reconstruction — because no competitor has designed for the field context (standing in a kitchen, between visits, one hand free) rather than for a clinician at a desk."*

Evidence across competitors

- AlayaCare: AI Form Assistant (voice-driven) exists but the mobile app crashes and loses data in field conditions — confirmed by AlayaCare's own FAQ and multiple user reviews; agencies require paper backup double-charting
- Heidi Health: ambient listening hardware just launched (March 2026) but designed for clinic/GP encounter settings, not home visit field conditions; no home-specific deployment model; Android reliability issues
- Lookout Way: field form completion is not supported; workers cannot complete forms in the system

The field context gap: No competitor has designed for the specific constraints of home care field documentation: no desk, one hand potentially occupied, unreliable connectivity, time pressure between visits, and the legal requirement for timestamped, field-specific detail that can withstand a coroner's inquiry.

Customer grounding

> *"The mg/mL thing still keeps me up at night. If the system had flagged it — 'this medication is usually measured in mL, did you mean mg?' — I would have caught it instantly. But there's no check. No validation. Just a free-text field and my thumbs on a phone screen." — David, HCN-002*

> *"Let me just talk. Let me say what happened and have the system figure out where it goes." — David, HCN-002*

> *"I spend more time writing about care than delivering it. By the time I sit down to type up today's visits, I've already forgotten half the details. I'm reconstructing, not documenting." — Maria, HCN-001*

> *"The solicitor asked me what time I gave Gerald his insulin. I said about 8:15. He said, 'Show me.' All my notes said was 'morning visit.'" — Priya, HCN-003 [coroner's inquest]*

Gap 5 — Continuity Failure from Soft-Preference Blindness

Gap 5 — Continuity Failure from Soft-Preference Blindness

Capability:	#5 Vacant Visit Smart Matching | **Type:** Utility | **Severity:** High

Opportunity statement

> *"Care coordinators lose clients, generate safety incidents, and spend 30–60 minutes per absence on phone calls when they fill vacant visits using availability and qualification matching alone — because the trust, preference, and continuity knowledge that prevents client distress and carer rejection exists only in coordinator memory and sticky notes, invisible to any scheduling system including AlayaCare's new Vacant Visit Agent."*

Evidence across competitors

- AlayaCare: Vacant Visit Scheduling Agent matches on caregiver availability and qualifications — no evidence of trust or preference layer; the "who does this patient trust" problem is explicitly unaddressed
- Lookout Way: rostering shows carer names and matching status; no soft-preference matching system identified
- Heidi Health: out of scope

The specific gap in AlayaCare's new agent: AlayaCare's Vacant Visit Scheduling Agent is a direct incumbent move in this space. But it optimises for operational efficiency (fill the slot) rather than care continuity (fill the slot with the right person for *this patient*). The first four of Angela's eleven phone calls found available staff. The next seven found the right person. AlayaCare's agent solves the first four calls. The next seven remain manual.

Customer grounding

> *"Last Tuesday. Jenny called in sick at six-thirty AM. She had three visits booked starting at eight. I made eleven phone calls. The first four were finding availability. The next seven were finding the right person." — Angela, CC-001*

> *"If I get hit by a bus tomorrow, half the knowledge about our clients walks out the door with me. It's on sticky notes and in my head." — Angela, CC-001*

> *"It has to know the soft stuff. Not just 'who's available' — 'who does this patient trust.'" — Angela, CC-001*

> *"We lost the Hendersons. Three different carers in two weeks. Her daughter said, 'My mother doesn't know who's walking through her door anymore.' They moved to another agency the following week." — Tom, CC-002*

> *"Nobody's father would sit in a chair waiting for someone who isn't coming." — Tom, CC-002*

Quantified cost of this gap

- Angela (CC-001): 45 min per incident × ~3 incidents/week = 2.25 hrs/week of reactive phone coordination
- Tom (CC-002): 30–60 min per absence; ~20% of vacant visits end in full cancellation
- Client loss: The Henderson family represents the compounding cost — lost revenue, reputational damage, and a distressed dementia patient — all attributable to continuity failure from soft-preference blindness

---

Gap 6 — The Invisible Care Plan

Gap 6 — The Invisible Care Plan

Capability:	#2 Care Plan Recommendation (patient visibility dimension) | **Type:** Utility + Trust | **Severity:** Medium

Opportunity statement

> *"Senior residents are excluded from visibility into their own care plans, clinical observations, and upcoming visit schedules — because all current platforms design for care teams, not for patients as active participants — leading to disengagement, information withholding, and missed self-management opportunities."*

Evidence across competitors

- AlayaCare: no patient portal; care plans are internal clinical documents
- Lookout Way: family app shows roster/schedule; no patient-facing care plan or clinical record access
- Heidi Health: notes are the clinician's record; patients receive no access by default

Customer grounding

> *"I don't know what my care plan says. Nobody has ever shown it to me. I assume there's a plan because people keep showing up, but what are they looking for? What am I supposed to be doing? I'd like to know." — Arthur, SR-002*

> *"Nobody asks me. I taught mathematics for thirty-five years. I can understand my own blood pressure numbers. I'd like to be a participant, not a subject." — Lin, SR-001*

> *"If I could ask my iPad, 'When is Maria coming?' or 'What did the doctor change last time?' — I'd use that." — Lin, SR-001*

---

8. Strategic Implication

8. Strategic Implication

The clearest market whitespace is not in clinical documentation (AlayaCare and Heidi are racing to own it) or operational scheduling efficiency (AlayaCare's Vacant Visit Agent is a strong incumbent move, but it solves the wrong half of the problem). The whitespace is in the **clinical intelligence layer that sits between the visit and the family** — a layer that:

- **Detects patterns nurses cannot see at scale** across multi-day visit gaps (Gap 1 — Capability #1)
- **Verifies what was actually ingested**, not just dispensed, and detects gaming (Gap 2 — Capability #1)
- **Captures field observations accurately** at the moment they happen, not reconstructed hours later (Gap 4 — Capability #3)
- **Fills vacant visits with the right person**, not just an available person, using soft preference knowledge that currently lives on sticky notes (Gap 5 — Capability #5)
- **Interprets and routes signals through clinical triage** before reaching families (Gap 3 — Capability #4)
- **Earns patient trust** by treating residents as participants rather than data subjects (Gap 6 — Capability #2)

No competitor owns this position across all six dimensions. AlayaCare is moving toward it from the agency-operations side; Lookout Way approaches it from the monitoring side; Heidi Health is orthogonal. None has closed the loop between patient trust, clinical interpretation, field-context capture, and family communication in a single coherent architecture.

The product that earns patient trust first will capture the most accurate data. The product with the most accurate data will have the best clinical outcomes. Clinical outcomes are the only durable moat in regulated healthcare.

9. Handshake to Next Skills

9. Handshake to Next Skills

Handshake ID	From this artifact	To skill	What transfers
HS-DISC-CGA-01	6 OST opportunity statements (Gaps 1–6)	`market-segmentation-deep-dive`	Score HCN, FC, CC, SR segments against each gap; CC segment must be scored against Gap 5 evidence (20% cancellation rate, Henderson client loss)
HS-DISC-CGA-02	Clinical intelligence layer whitespace (Section 8)	`opportunity-solution-tree`	Use as the Desired Outcome node
HS-DISC-CGA-03	AI Trust Architecture constraint (Section 6)	`ethics-trust-mapping`	Defines Red/Yellow zone data types; wellness performance as design risk
HS-DISC-CGA-04	Notification routing as architectural rule	`agentic-safety-discovery`	Nurse-first routing must be enforced at system level, not configuration
HS-DISC-CGA-05	Medication adherence market gap (Gap 2 + Section 4)	`brainstorm-ideas-new`	No existing solution to reference; requires first-principles ideation
HS-DISC-CGA-06	Capability Coverage Map (Section 5)	`opportunity-solution-tree`	One opportunity node per capability; Gaps 1–6 map to Capabilities 1–5

Artifact 3 amended. 6 OST opportunity statements. All 5 product capabilities covered. AI Trust Architecture repositioned as cross-cutting design constraint. CC persona evidence surfaced in Gap 5. Next step: `market-segmentation-deep-dive` *— score HCN, FC, CC, and SR segments against Gaps 1–6 to determine Beachhead Market and Right to Win.*

Home-Care-AI · Artifact 3 · Competitive Gap Analysis · 2026-03-26